

CLINICAL FINDINGS

Approach to the Patient

The approach to a new patient with cutaneous dermatomyositis (DM) involves a comprehensive evaluation of both skin and muscle involvement, as well as potential systemic associations, including malignancy.

History

In 60% of patients with classic DM, cutaneous lesions and muscle weakness present simultaneously. In 30%, skin findings precede muscle weakness, while in 10%, muscle weakness develops before the appearance of skin lesions. The onset of cutaneous disease is often accompanied by pruritus and/or a burning sensation in the skin. Photosensitivity to sunlight or artificial ultraviolet light is commonly reported.

Muscle weakness typically affects the proximal muscles of the shoulder and hip girdles. Clinically, this manifests as difficulty raising the arms above the head or rising from a seated position. Muscle pain and tenderness may accompany weakness.

Cutaneous Lesions

Hallmark Skin Lesions

The hallmark cutaneous manifestations of DM are listed in Table 157-4. No recognizable differences in clinical, histopathologic, or immunopathologic features have been reported between classic DM and clinically amyopathic dermatomyositis (CADM).

Some lesions are pathognomonic for DM, including: - **Gottron sign** – erythematous or violaceous macular eruption over the extensor surfaces of the

interphalangeal joints. - **Gotttron papules** – discrete, violaceous, often scaly papules over the knuckles.

Other highly characteristic findings include: - **Heliotrope rash** – periorbital, confluent, macular, violaceous erythema and edema. - **Periungual telangiectasias** – visible telangiectatic vessels around the nail folds, often associated with dystrophic cuticles.

The primary skin lesion in DM is a symmetric, confluent, macular, violaceous erythema that is often pruritic. It typically affects the following areas: - Extensor aspects of fingers, hands, and forearms - Arms, deltoid regions, posterior shoulders, and neck (**shawl sign**) - V-area of the anterior neck and upper chest (**V-sign**) - Central face, periorbital regions, forehead, and scalp - Lateral hips and thighs (**holster sign**)

Facial involvement may mimic seborrheic dermatitis, particularly when nasolabial folds are affected.

Other cutaneous features include: - **Poikiloderma atrophicans vasculare** (also seen in poikilodermatomyositis; may overlap with cutaneous T-cell lymphoma) - **Calcinosis cutis** (also observed in scleroderma)

Overlap syndromes, such as **sclerodermatomyositis** (features of both DM and scleroderma), can occur in the same individual.

Table 157-5 outlines the characteristic anatomic distribution of hallmark skin lesions in DM. There is limited data on the prevalence of involvement in specific areas. Notably, dermatologists have reported Gotttron papules more frequently than heliotrope rash, which is traditionally emphasized as the hallmark lesion in non-dermatology literature.

The violaceous coloration of skin inflammation in lightly pigmented individuals is a striking and early feature of DM. This helps differentiate it from cutaneous lupus erythematosus (LE), where violaceous discoloration, if present, typically appears later in the disease course.

The violaceous hue may result from edema displacing hyperemic superficial blood vessels deeper into the dermis, altering light absorption. Alternatively, it may reflect deoxygenation of blood in dilated dermal capillaries.